

Statement of Best Practice

for

Critical Illness Cover

Association of British Insurers

May 2002

Introduction

This Statement of Best Practice for Critical Illness Cover aims to help consumers understand and compare critical illness policies by standardising the following:

- The description of Critical Illness Cover in Key Features
- The use of Generic Terms
- The use of Model Wordings for Core and Additional Conditions and Model Exclusions

This Statement of Best Practice falls under the ABI Code of Life (Non Investment) Business and is mandated for members of the Association of British Insurers (ABI) offering critical illness cover. It replaces the earlier Statement of Best Practice for Critical Illness Cover dated April 1999.

Insurers should adopt the changes as soon as is practical but must do so by no later than the end of May 2003.

The Association of British Insurers is the trade association for insurance companies in the United Kingdom. We represent over 400 insurance companies, which provide over 96% of the insurance business in the UK. We represent insurance companies to the Government, and to regulatory and other agencies, and provide a wide range of services to our members.

Contents

1. General Principles
2. Use of Key Features
3. Generic Terminology
4. Core & Additional Conditions
5. Model Exclusions
6. Total Permanent Disability
7. Review Process

Appendix A: Example Key Features

1 General Principles

- 1.1 This Statement of Best Practice applies to critical illness cover insurers who are members of the ABI and is based on the following principles. Critical illness cover means cover which pays out on a diagnosis of a listed critical illness. The list of illnesses must include cancer, heart attack and stroke.

Clarity

- 1.2 Wherever possible to aid consumer understanding, the wording of Key Features will be in "Plain English" provided that this does not dilute or conflict with the meaning.
- 1.3 The heading (for Core and Additional Conditions and Model Exclusions) forms part of the model wording. If an insurer includes an optional age limit in a particular condition, this should appear in the heading of the condition.
- 1.4 All model wordings should be as robust as possible in differentiating between what is, and is not, covered to:
- 1.4.1 Create a clear expectation of the scope and limitations of the cover.
 - 1.4.2 Allow valid claims to be paid promptly.
 - 1.4.3 Minimise the number of disputed claims to avoid disappointment.
- 1.5 All conditions should be listed in alphabetical order (within sections as appropriate).

Generic Terminology

- 1.6 For clarity, the generic terminology should be used in all cases where the appropriate generic terms apply. Other terms to describe these features should not be used.
- 1.7 The generic terms are described only as far as required to establish the context. Insurers may use these descriptions as definitions if they wish. Alternatively, insurers may use their own definitions (provided they suit the context) or may use the terms without defining them.

Key Features

- 1.8 The Key Features requirements set out in this document are in addition to (and, in the event of a conflict, are overruled by) any other regulatory, legal, 3rd Life Directive and product specific requirements for Key Features.
- 1.9 The Key Features format is intended to ensure that critical illness cover is described in the same way, regardless of the underlying product type. This is intended to allow consumers to compare the critical illness cover of different insurers, accepting that the underlying products may be different.
- 1.10 All long-term products featuring critical illness cover should use the standard Key Features format – regardless of whether the sale of the product is regulated by the FSA. This includes (but is not limited to) the following product types where critical illness cover is included as a standard feature or as an optional benefit:
- 1.10.1 Endowment
 - 1.10.2 Whole of Life
 - 1.10.3 Term Assurance
 - 1.10.4 Stand Alone Critical Illness Cover
- 1.11 Insurers should give Key Features documents to enquirers and potential customers (via intermediaries as appropriate) at the earliest opportunity to allow them to make meaningful product comparisons before purchase.
- 1.12 The format is based on the principle that critical illness cover is not usually a product in its own right, but is more frequently a benefit added to an underlying product type.

Policies with Combined Benefits

- 1.13 Where critical illness cover is offered on product types that are not regulated by the FSA where the Key Features do not allow the description of critical illness cover to be included (eg a policy with both income protection and critical illness cover), insurers may use a separate Key Features for the critical illness cover.

Group Critical Illness Cover

- 1.14 This Statement of Best Practice applies to individual long-term critical illness contracts. Insurers of group critical illness cover should follow the provisions relating to Generic Terminology and model wordings for Core and Additional Conditions and Model Exclusions.

Implementation

- 1.15 The provisions apply to new policies effected on or after the implementation date adopted by the insurer. An increment or increase to an existing policy effected after that date as a new policy may be excluded if it mirrors the original contract.
- 1.16 Insurers do not have to apply revised definitions to existing in-force policies.

Categorising Model Wordings

- 1.17 Model wordings for medical conditions, surgical procedures, disabilities and policy exclusions are divided into "Core Conditions", "Additional Conditions" and "Model Exclusions". The constituents will be determined at each review (and subsequently published) based on the following:
- 1.17.1 "Core Conditions" are the conditions that, together, account for the majority of critical illness claims. They will normally be any existing Core Conditions and additionally those included on at least 95% of policies on the market at the time of the review, and account for at least 1% of male or female critical illness industry claims.
- 1.17.2 "Additional Conditions" are other (non-core) conditions for which there is a model wording available. They will normally be those conditions included in the policy of at least 75% of critical illness cover policies on the market at the time of the review.
- 1.17.3 "Model Exclusions" are the policy exclusions and limitations where a model wording is available. These will normally be any existing Model Exclusions and additionally any exclusions and limitations included in the policy of at least 50% of critical illness cover policies on the market at the time of the review.

Basis of Standard for New Model Wordings

- 1.18 All model wordings are on the basis of an "appropriate minimum standard". Insurers are free to offer additional cover by including other conditions or by offering additional cover as described in Principle 1.24 below.
- 1.19 Where alternative wordings are used in the market to describe different levels of cover, the model wording will normally reflect the most robust definition, provided that this is appropriate as a minimum standard (i.e. offers meaningful cover).
- 1.20 Where several wordings are used in the market to describe essentially the same level of cover, the model wording will be based on the most robust recommended wording of the leading critical illness cover reassurers (as a starting point).

Overlaps in Conditions Covered

- 1.21 Where some insurers combine several conditions under one heading, others choose to offer the conditions under several headings, the model wordings will normally be based on the individual (as opposed to combined) definitions. Insurers who choose to combine the wordings into a single definition would not be deemed to be using the model wording as the heading forms part of the model wording.

- 1.22 No model wording will be produced for conditions that wholly overlap with another Core or Additional Condition (e.g. no Leukaemia model wording will be available as this is wholly covered under Cancer).

Use of Model Wordings

- 1.23 Insurers are free to omit any condition or exclusion and may include any other conditions or exclusions as they see fit. While insurers are free to decide on the conditions and exclusions applicable to their products, where a model wording is available, it should be used.
- 1.24 Insurers will be deemed to be using the model wording (for a condition or exclusion) where it is modified to provide at least equivalent cover in the following ways:
- 1.24.1 By using the model wording and showing separately the additional cover offered.
- 1.24.2 By omitting a specific limitation or exclusion in the model wording for any condition or exclusion, while leaving the remaining words unchanged. For example, this could be by omitting the words "All forms of lymphoma in the presence of any Human Immunodeficiency Virus." from the model wording of Cancer or by omitting "off-piste skiing" from the Hazardous sports & pastimes exclusion.
- 1.24.3 By using or omitting specified optional wording (i.e. for 'loss of limbs' or age limits).
- 1.25 Insurers should set out their specific claim requirements in their policy, for example in a general heading along the following lines:

Example Policy Condition

All diagnoses and medical opinions must be given by a medical specialist who:

- is a Consultant at a hospital in the UK;
- is acceptable to our Chief Medical Officer; and
- is a specialist in an area of medicine appropriate to the cause of the claim.

Reviews of Existing Model Wordings

- 1.26 No changes should be recommended to any existing agreed wordings without both the following:
- 1.26.1 A clear issue that has resulted (or is expected to result) in industry-wide problems for customers and/or insurers.
- 1.26.2 Agreement (with a clear rationale) that the proposed wording change will address the issue.

2 Use of Key Features

General

- 2.1 Potential customers should receive Key Features at the earliest opportunity to allow them to compare products before purchase.
- 2.2 In the following guidelines, the use of *[square brackets in italics]* indicates that the wording of Key Features is illustrative of the wording that should be used and may be amended from that shown. Otherwise, the actual wording shown should be used. Insurers are free to adopt any layout for their Key Features to suit their corporate style (eg 2 columns).

Front Page Headings

- 2.3 The "front-page headings" (Its Aims, Your Commitment and Risk Factors) are already mandated for regulated business. This is extended under this Statement of Best Practice to non-regulated business. Under each of these headings, the following should be used to describe these elements of the critical illness cover, in addition to the requirements for other benefits.
- 2.4 Under "Its Aims" there should be a statement describing the cover or option, depending on the context. Where benefits are payable once, on death or earlier critical illness, the wording should make it clear that only one payment is made as follows:

Its aims

[The plan aims to do the following.]

- *[To pay a guaranteed lump sum if {you die / an insured person dies} or {is/are} diagnosed with a terminal illness during the period of cover.]*
- *[To pay the guaranteed lump sum if, during the period of cover, {you are / an insured person is} diagnosed with a specified critical illness, as long as you choose critical illness cover.]*

- 2.5 Under "Your Commitment", any obligations on the customer that are required to maintain the cover unchanged should be stated. For regular premium contracts this should include a statement about the duration of premiums. If increasing premiums are a feature of the policy, the basis should be stated. Any requirements to notify the insurer of any changes in personal circumstances and the potential consequences should also be stated in this section as follows:

Your commitment

[You must do the following.]

- *[Pay a regular premium during the period of cover.]*
- *[Tell us if {you / an insured person} {change your / changes their} job.]*
- *[Tell us if {you start / an insured person starts} living abroad.]*
- *[Tell us if {you / an insured person} starts or stops smoking.]*

- 2.6 Under "Risk Factors" there should be a statement warning that the policy will not pay out on non-disclosure, or if the cause of a claim is related to an exclusion. If appropriate, a warning about increases in premiums should be included if premiums could increase (for example following a policy review) together with the reasons for the potential increase (such as age, claims experience, investment performance, charges, lifestyle changes as appropriate). There should be an appropriate statement to warn that cover could stop when premiums cease. These should be as follows:

Risk Factors

[The plan carries the following risks.]

- *[If you do not provide any information we ask for, we will not pay out under the plan.]*
- *[If you stop paying your premiums your cover will end after x days.]*
- *[When we review your policy after x years we may increase your premium if our experience of claims shows that we should increase all planholders' premiums. We will tell you if we are going to do this.]*
- *[We will not pay out in the circumstances described under the heading [{"exclusions"} / 'When will the plan not pay out'] on page x.]*

Description of Critical Illness Cover

- 2.7 In a section of the Key Features, insurers should show what conditions are included in the critical illness cover using the relevant sub-sections below as appropriate (conditions covered, optional conditions, child cover and further information). The following guidelines apply:
- 2.7.1 Insurers should include this section in any appropriate place within the main body of the Key Features.
 - 2.7.2 The opening question should refer to what is included in "critical illness" or "critical illness cover".
 - 2.7.3 The main introduction must be unambiguous that the list of conditions is an exhaustive list. Phrases such as "We cover the following:" and "The following are included:" should not be used. "The complete list of conditions we cover is as follows." or "We cover only the following conditions." would be appropriate.
 - 2.7.4 Insurers should list all the conditions covered.
 - 2.7.5 If an insurer includes age limits in certain conditions, these should be shown in the heading of the condition.
 - 2.7.6 If insurers offer different levels of cover or certain extra conditions at extra cost, these should be shown separately.
 - 2.7.7 Within each section, conditions should be listed in alphabetical order.
 - 2.7.8 Insurers may describe child critical illness cover in this section, or elsewhere, if applicable.
 - 2.7.9 The "further details" should include where the full definitions and claims requirements (such as medical evidence) may be found.
 - 2.7.10 The answers should be in the format shown below and following the above guidelines.

[Which critical illnesses are covered?]

- *[The complete list of conditions we cover is shown below.]*
 - *[Alzheimer's disease before age 60*
 - *aorta graft surgery*
 - *benign brain tumour*
 - *blindness]*
 - *cancer*
 - *[coma*
 - *coronary artery by-pass surgery*
 - *deafness]*
 - *heart attack*
 - *[heart valve replacement or repair*
 - *HIV from a blood transfusion*
 - *kidney failure*
 - *loss of limbs*
 - *loss of speech*
 - *major organ transplant*
 - *motor neurone disease before age 60*
 - *multiple sclerosis*
 - *paralysis / paraplegia*
 - *Parkinson's disease before age 60]*
 - *stroke*
 - *[terminal illness*
 - *third degree burns*
 - *total permanent disability before age 60]*
- **Please remember that the heading of each critical illness is only a guide to what is covered.** For example, some types of cancer are not covered. Further details of how we will consider your claim, including the full definitions we will use and the evidence we will need, are given in the *[definitions guide and policy document.]* You can ask us for *[{a} {sample} {copy / copies} of {this / these}].*
- *[Children are also covered for the conditions listed above. Child cover applies from the date each child turns three and ends when they become 18, as long as the plan is in force. The most we will pay for a claim for a child is £10,000 and we will only pay one claim for each child. Claims for children do not affect your cover.]*
- *[You may also apply to be covered for {optional benefits}. Your personal illustration shows if this {optional benefit} is included in your plan and, if so, the cost.]*

Description of Exclusions and Limitations to Critical Illness Cover

- 2.8 In a section of the Key Features, insurers should give a summary of the exclusions and limitations. This should include whichever of the following are applicable in the format shown below:
- 2.8.1 The claims notification period if exceptionally there is a specified fixed period after which a claim will not be paid (see ABI Statement of Long-Term Insurance Practice).
 - 2.8.2 The survival period (normally for stand-alone critical illness cover).
 - 2.8.3 The headings of any model exclusions used.
 - 2.8.4 Any other limitations or exclusions which could prevent the policy paying out.
 - 2.8.5 A statement regarding individual exclusions applied as part of the underwriting process.
 - 2.8.6 Any restrictions that apply to child cover.
 - 2.8.7 Details of where further information may be found.
- 2.9 The Key Features should show all exclusions and limitations. For model exclusions, only the headings need be shown with a reference to where the full wording may be found (as with the conditions covered).
- 2.10 The following introductory question should be included as shown. It should be shown with equal prominence to the description of the critical illness cover. Exclusions for other benefits (e.g. life cover or waiver of premium benefit) may be included under the same heading. The format of the answers should be as follows:

[When will the plan / policy not pay out?]

[We will not pay benefit for a critical illness in the following circumstances.]

- *[If you do not make a claim within x months of the critical illness being diagnosed.]*
- *[If the insured person dies within x days of the condition being diagnosed.]*
- *[If your claim arises within three months of you starting up a plan that has previously ended.]*
- *[If the cause of the claim results from aviation, criminal acts, drug abuse, failure to follow medical advice, hazardous sports and pastimes, HIV/AIDS, self-inflicted injury or war and civil commotion.]*
- *[If the claim is for a total permanent disability from a cause that arose while the insured person was living abroad.]*
- *[If the claim is for a total permanent disability and you did not tell us about the insured person changing their job.]*
- *[If the claim is for a child diagnosed with a critical illness and:*
 - *the condition runs in the family; or*
 - *was present at birth; or*
 - *the symptoms first arose before the child was covered.]*

Full details of what is covered, and any limits to the cover, are given in the *[definitions guide and policy document]*. You can ask us for *[(a) {sample} {copy / copies} of {this / these}]*.

We may *[also]* apply specific exclusions when we accept your policy. These will be shown *[in your acceptance letter and policy schedule.]*

Further Information

- 2.11 The Key Features should inform customers of the availability of the ABI Guide to Critical Illness Cover, which may be obtained from the insurer or the ABI.
- 2.12 Insurers should clearly set out their complaints procedure and, where appropriate, provide contact details for the Financial Ombudsman Service (FOS).
- 2.13 Insurers should state that their Key Features documents comply with this Statement of Best Practice.

Customer Specific Key Features

- 2.14 The Customer Specific Key Features should clearly state the basis of the premium. This should make it clear to potential customers whether the premium is level or increasing, guaranteed or reviewable (and if so, when the reviews take place). If the premium is on a level basis, this should be stated. This information should be shown prominently.

3 Generic Terms

- 3.1 When generic terms are used they should have the meanings shown and other terms should not be used in their place. This is to ensure that the terms always have the same meaning.
- 3.2 Not all the terms will apply to the critical illness cover contained in all products. Insurers should only use those terms that are applicable.
- 3.3 The Generic Terms and associated descriptions as set out below are intended to establish the context in which each term should be used. Insurers are free to use them as definitions, or as part definitions, or may use alternative definitions, provided these suit the context.
- 3.4 The terms "critical illness cover" and "critical illness" apply to the type of cover. Insurers are free to use marketing names for their products (eg Living Plan etc) and cover (eg Living Benefit), provided that the cover is described either as "critical illness cover" or by using the words "critical illness".

The Generic Terms

- 3.5 **Additional conditions**
The conditions, apart from the core conditions and total permanent disability, which model wordings are available for.
- 3.6 **Assessment period**
The period during which we will assess a condition before we make a decision on whether or not to accept a claim. The assessment period will not normally be longer than 12 months, as long as we have all the evidence we need. Also, the assessment period should only apply to claims for the conditions which must be permanent for cover to apply.
- 3.7 **Core conditions**
The core conditions are cancer, coronary artery by-pass surgery, heart attack, kidney failure, major organ transplant, multiple sclerosis and stroke.
- 3.8 **Critical Illness Cover**
Cover which pays out on a diagnosis of a listed critical illness. The list of illnesses must include cancer, heart attack and stroke.
- 3.9 **Deferred period**
The period during which an insured person must be ill or disabled before we will pay any benefit.
- 3.10 **Irreversible**
Cannot be cured by medical treatment and/or surgical procedures used by the National Health Service in the UK at the time of the claim.
- 3.11 **Model exclusions**
The exclusions to cover for which model wordings are available.
- 3.12 **Permanent**
Expected to last throughout the insured person's life, irrespective of when the cover ends or the insured person retires.
- 3.13 **Survival period**
The period after an insured event covered by the plan that the insured person has to survive before we will accept a claim. A survival period normally applies to plans providing critical illness cover only or where the level of death benefit is not the same as the benefit due for a critical illness.

4 Core & Additional Conditions

- 4.1 Insurers are free to decide on the conditions applicable to their products.
- 4.2 Insurers may offer additional cover whilst still being deemed to use the model wordings, subject to doing so as set out above (see General Principles).
- 4.3 The heading forms part of the model wording. However, insurers may choose to include a qualifying phrase in a heading, providing this is included after the heading shown in the model wording so that the condition retains its alphabetical place in any listing. For example, this could be by replacing the heading "cancer" with the revised heading "cancer (most malignant types)".
- 4.4 The use of *[square brackets and italics]* means that the wording shown may vary as follows:
- 4.4.1 The age limits for motor neurone disease and Parkinson's disease are optional. However, if an age limit is included in the definition, it should also be included in the heading (as shown in the policy, key features and other material).
- 4.4.2 Insurers may choose whether to use "elbow / knee" or "ankle / wrist" for loss of limbs.
- 4.4.3 The AIDS exclusion shown in the model wording of terminal illness should not be used in the case of accelerated cover.

The Core Conditions

- 4.5 Cancer
- Any malignant tumour characterised by the uncontrolled growth and spread of malignant cells and invasion of tissue. The term cancer includes leukaemia and Hodgkin's disease but the following are excluded:
- All tumours which are histologically described as pre-malignant, as non-invasive or as cancer in situ.
 - All tumours of the prostate unless histologically classified as having a Gleason score greater than 6 or having progressed to at least TNM classification T2N0M0.
 - All forms of lymphoma in the presence of any Human Immunodeficiency Virus.
 - Kaposi's sarcoma in the presence of any Human Immunodeficiency Virus.
 - Any skin cancer other than invasive malignant melanoma.
- 4.6 Coronary artery by-pass surgery
- The undergoing of open heart surgery on the advice of a Consultant Cardiologist to correct narrowing or blockage of one or more coronary arteries with by-pass grafts but excluding balloon angioplasty, laser relief or any other procedures.
- 4.7 Heart attack
- The death of a portion of heart muscle, due to inadequate blood supply, that has resulted in all of the following evidence of acute myocardial infarction:
- typical chest pain;
 - new characteristic electrocardiographic changes;
 - the characteristic rise of cardiac enzymes, troponins or other biochemical markers;
- where all of the above shows a definite acute myocardial infarction. Other acute coronary syndromes, including but not limited to angina, are not covered under this definition.
- 4.8 Kidney failure
- End stage renal failure presenting as chronic irreversible failure of both kidneys to function, as a result of which either regular renal dialysis or renal transplant is initiated.
- 4.9 Major organ transplant
- The actual undergoing as a recipient of, or inclusion on an official UK waiting list for, a transplant of a heart, liver, lung, pancreas or bone marrow.

4.10 Multiple sclerosis

A definite diagnosis by a Consultant Neurologist of Multiple Sclerosis which satisfies all of the following criteria:

- There must be current impairment of motor or sensory function, which must have persisted for a continuous period of at least six months.
- The diagnosis must be confirmed by diagnostic techniques current at the time of the claim.

4.11 Stroke

A cerebrovascular incident resulting in permanent neurological damage. Transient Ischaemic Attacks are specifically excluded.

The Additional Conditions

4.12 Aorta graft surgery

Undergoing surgery for disease of the aorta needing excision and surgical replacement of a portion of the diseased aorta with a graft. For this definition, aorta means the thoracic and abdominal aorta but not its branches.

4.13 Benign brain tumour

A non-malignant tumour in the brain resulting in permanent deficit to the neurological system. Tumours or lesions in the pituitary gland are not covered.

4.14 Blindness

Total permanent and irreversible loss of all sight in both eyes.

4.15 Coma

A state of unconsciousness with no reaction to external stimuli or internal needs, persisting continuously with the use of life support systems for a period of at least 96 hours and resulting in permanent neurological deficit. Coma secondary to alcohol or drug misuse is not covered.

4.16 Deafness

Total permanent and irreversible loss of all hearing in both ears.

4.17 Heart valve replacement or repair

Undergoing open heart surgery from medical necessity to replace or repair one or more heart valves.

4.18 Loss of limbs

The permanent physical severance of two or more limbs from above the *[elbow/wrist]* or *[knee/ankle]* joint.

4.19 Loss of speech

Total permanent and irreversible loss of the ability to speak as a result of physical injury or disease.

4.20 Motor neurone disease [before age x]

Confirmation by a Consultant Neurologist of a definite diagnosis of Motor Neurone Disease *[before age x]*.

4.21 Paralysis / paraplegia

Total irreversible loss of muscle function or sensation to the whole of any two limbs as a result of injury or disease. The disability must be permanent and supported by appropriate neurological evidence.

4.22 Parkinson's disease [before age x]

Confirmation by a Consultant Neurologist of a definite diagnosis of Parkinson's Disease *[before age x]*. Parkinson's Disease secondary to alcohol or drug misuse is not covered.

4.23 Terminal illness

Advanced or rapidly progressing incurable illness where, in the opinion of an attending Consultant and our Chief Medical Officer, the life expectancy is no greater than 12 months. *[AIDS is specifically excluded and not covered under this definition.]*

4.24 Third degree burns

Third degree burns covering at least 20% of the body surface area.

5 Model Exclusions

- 5.1 Insurers are free to omit any of the model exclusions and may include additional exclusions.
- 5.2 Insurers may offer additional cover whilst still being deemed to use the model wordings, subject to doing so as set out above (see General Principles).
- 5.3 As with core and additional conditions, the headings form part of the model wording.
- 5.4 All exclusions and limitations (not only model exclusions) should be contained in one section in the policy (and Key Features as set out above).
- 5.5 Insurers may define European Union as a list of EU Countries as at the start of the contract.
- 5.6 Insurers should state which exclusions apply to which conditions in their policy (and other benefits as appropriate, e.g. waiver of premium benefit) and should use an introductory policy wording to suit their individual policy wording style (see the example below).

Example introductory wording for model exclusions

"We will not pay a critical illness claim if it is caused directly or indirectly from any of the following:"

The model exclusions

- 5.7 **Aviation**
Taking part in any flying activity, other than as a passenger in a commercially licensed aircraft.
- 5.8 **Criminal acts**
Taking part in a criminal act.
- 5.9 **Drug abuse**
Alcohol or solvent abuse, or the taking of drugs except under the direction of a registered medical practitioner.
- 5.10 **Failure to follow medical advice**
Unreasonable failure to seek or follow medical advice.
- 5.11 **Hazardous sports and pastimes**
Taking part in (or practising for) boxing, caving, climbing, horse-racing, jet skiing, martial arts, mountaineering, off-piste skiing, pot-holing, power-boat racing, under-water diving, yacht racing or any race, trial or timed motor sport.
- 5.12 **HIV/AIDS**
Infection with Human Immunodeficiency Virus (HIV) or conditions due to any Acquired Immune Deficiency Syndrome (AIDS).
- 5.13 **Living abroad**
Living outside of the European Union for more than 13 consecutive weeks in any 12 months.
- 5.14 **Self-inflicted injury**
Intentional self-inflicted injury.
- 5.15 **War and civil commotion**
War, invasion, hostilities (whether war is declared or not), civil war, rebellion, revolution or taking part in a riot or civil commotion.

6 Total Permanent Disability

- 6.1 There are a number of definitions used for Total Permanent Disability. For example, "total disability" may be measured by assessing the person's ability to perform certain of the following:
- 6.1.1 The insured person's "Own Occupation".
 - 6.1.2 "Suited Occupations".
 - 6.1.3 "Any Occupation".
 - 6.1.4 A number of specified "Activities of Daily Living".
 - 6.1.5 A number of specified "Functional Ability Tests".

Change of Occupation

- 6.2 Depending on whether the definition used relies on the occupation of the person covered, this occupation may be disclosed and underwritten at the outset of the policy. Where this applies, subsequent changes of occupation may vary the initial underwriting assessment. Insurers normally deal with this issue in one or a combination of the following ways:
- 6.2.1 Notification is required. The new occupation is re-underwritten and the terms of the contract are adjusted if necessary. The customer's ability to perform an occupation (or one that is suited, depending on the definition) is judged against the occupation most recently notified (if all changes have been notified as required).
 - 6.2.2 Notification is not required. The customer's ability to perform an occupation (or one that is suited, depending on the definition) is judged against the occupation being followed immediately before the claim. In this case, the insurer accepts the risk of changes in occupation.
 - 6.2.3 Alternatively, the insurer judges the claim against the occupation declared in the application, regardless of subsequent changes.
- 6.3 "Occupation" generally means a trade, profession or type of work undertaken for profit or pay. It is not a specific job with any particular employer and is independent of location.

Guidelines

- 6.4 Insurers are free to use any one or more definitions of disability they wish, including but not limited to those shown in 6.1 above.
- 6.5 Insurers should make it clear in Key Features which procedure is adopted in relation to change of occupation together with the potential consequences.
- 6.6 If insurers require notification of changes in occupation, they should periodically remind the policyholder.

7 Review Process

- 7.1 This document has been prepared by following the normal process that is proposed for future reviews, following the principles above.
- 7.2 Industry-wide provisions, such as the Key Features format set out in this document, and model wordings based on market practice, legislation and medical science should be reviewed regularly to ensure that they remain up to date. The ABI will therefore make provision for reviews as follows.

Types of Review

- 7.3 There will be 2 types of review: Full Reviews (normally every 3 years) and Intermediate Reviews. The ABI Critical Illness Working Party will carry out all Full and Intermediate Reviews.

Full Reviews

- 7.4 Full Reviews will be carried out every 3 years unless the ABI Critical Illness Working Party decides not to carry out the Full Review in which case it will be deferred for up to 1 year.
- 7.5 At a Full Review, the scope will be to review the Key Features format and all model wordings in line with the principles set out above. This will take into account changes in medical science, relevant "events" (such as changes in legislation since the last Full or Intermediate Review), experience, available research, and current market practice. The process should use the expertise of Critical Illness Working Party members and appropriate liaisons.
- 7.6 At a Full Review, the recommendations will include a recommended process for implementing any changes and the review process itself should be reviewed and any changes put forward for the next review.

Intermediate Reviews

- 7.7 An Intermediate Review may be held where a compelling issue is raised that is, for example, of a legal or regulatory nature. Less compelling issues will normally be dealt with at a Full Review.
- 7.8 The scope of Intermediate Reviews will be limited to the agreed impact of the issue raised and recommendations for implementing any changes. Other issues will be outside the scope of the Intermediate Review.
- 7.9 The normal process for establishing an Intermediate Review will be:
 - 7.9.1 An issue is raised with the ABI Critical Illness Working Party.
 - 7.9.2 The Working Party decides on whether to carry out the Intermediate Review - otherwise the issue is recorded for the next Full Review.